

Targeted therapy and immunotherapy for womb cancer

Targeted therapy and immunotherapy may be used to help control womb cancer and slow down the growth.

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Targeted therapy and immunotherapy for womb cancer

Targeted therapy and immunotherapy may be used to treat womb cancer that has:

- come back (recurrent cancer)
- spread to other parts of the body (metastatic cancer).

Your cancer doctor will decide if targeted therapy or immunotherapy might be suitable for you. They will look at the [type](#), [stage](#), [grade](#), [genetic changes](#) and [risk group](#) of your cancer.

They may be given as part of a [clinical trial](#).

Targeted therapy for womb cancer

Targeted therapies are drugs that find and attack cancer cells. Targeted therapy can help control the cancer and slow down the growth.

Your cancer doctor may use a targeted therapy called [lenvatinib](#) (Lenvima®). This is given as capsules you can take at home. It can be given in combination with an immunotherapy drug called [pembrolizumab](#) (Keytruda®) which is given into a vein as an outpatient. We have more information about these [targeted therapy drugs](#).

We also have more information about the different targeted therapy drugs and [combination treatments](#).

Immunotherapy for womb cancer

Immunotherapy uses your immune system to find and attack cancer cells.

Your cancer doctor may use the following immunotherapy drugs:

- [dostarlimab](#) (Jemperli®)
- [pembrolizumab](#) (Keytruda®).

Pembrolizumab can also be given with a targeted therapy called [lenvatinib](#) (Lenvima®) which comes as capsules you take at home.

You usually have immunotherapy as an outpatient. Specialist chemotherapy nurses give immunotherapy and other anti-cancer drugs.

The drugs used for womb cancer are given into a vein (intravenously) similarly to [chemotherapy](#).

Side effects of immunotherapy

Immunotherapy can cause the immune system to become more active. That means it is better at finding and attacking cancer cells. But immunotherapy can also cause unwanted side effects. These are very different to the side effects of other cancer treatments, such as chemotherapy, radiotherapy, targeted therapy or hormonal therapy. Some people do not have many side effects, but side effects can be serious if they happen.

If immunotherapy makes the immune system too active it can cause inflammation in the body. This can be serious. Sometimes this can happen weeks, months or even more than a year after you finish immunotherapy. Your cancer doctor or specialist nurse can tell you more.

We have more information about having [immunotherapy](#).

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our womb cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

Oaknin, A. et al. Endometrial cancer: ESMO Clinical Practice Guideline for diagnosis, treatment and follow-up. Annals of Oncology, Volume 33, Issue 9, 860 - 877
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Berek JS, Matias-Guiu X, Creutzberg C, et al. FIGO staging of endometrial cancer: 2023. Int J Gynecol Obstet. 2023; 162: 383-394. doi:10.1002/ijgo.14923 (accessed January 2024).

Suspected cancer: recognition and referral. NICE guideline [NG12] Published: 23 June 2015 Last updated: 02 October 2023 [Overview](#) | [Suspected cancer: recognition and referral](#) | [Guidance](#) | [NICE](#) (accessed January 2024).

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.



Womb cancer forum

An anonymous network of people affected by womb cancer which is free to join. Share experiences, ask questions and talk to people who understand.

Get help